



Bill Number: H.B. 2195

Gowan Floor Amendment

Reference to: HEALTH AND HUMAN SERVICES

Committee amendment

Amendment drafted by: Leg. Council

FLOOR AMENDMENT EXPLANATION

- Allows the Department of Health Services to initiate a complaint investigation of a nursing care institution for incidents that occurred more than 12 months before the complaint if the complaint involves an abuse allegation.

GOWAN FLOOR AMENDMENT
SENATE AMENDMENTS TO H.B. 2195
(Reference to HEALTH AND HUMAN SERVICES Committee amendment)

Amendment instruction key:

[GREEN UPPERCASE UNDERLINING IN BRACKETS] indicates that the amendment is adding text to statute or previously enacted session law.
[Green lowercase underlining in brackets] indicates that the amendment is adding text to new session law or is restoring previously stricken text to existing statute.
~~[GREEN UPPERCASE STRIKEOUT IN BRACKETS]~~ indicates that the amendment is removing new text from statute or previously enacted session law.
~~[Green lowercase strikeout in brackets]~~ indicates that the amendment is removing text from existing statute, previously enacted session law or new session law.
<<Double green carets enclosing an entire section>> indicates that the amendment is adding the section to the bill.
~~<<Green strikeout with double green carets enclosing an entire section>>~~ indicates that the amendment is removing the section to the bill.
{[ORANGE UPPERCASE UNDERLINING IN DOUBLE CURLY BRACKETS]} indicates that the amendment to an amendment is adding text to statute or previously enacted session law.
{[Orange lowercase underlining in double curly brackets]} indicates that the amendment to an amendment is adding text to new session law or is restoring previously stricken text to existing statute.
~~{[ORANGE UPPERCASE STRIKEOUT IN DOUBLE CURLY BRACKETS]}~~ indicates that the amendment to an amendment is removing new text from statute or previously enacted session law.
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<<Double orange underlined carets enclosing an entire section>> indicate that the amendment to an amendment is adding the section to the bill.
~~<<Orange strikeout with double orange underlined carets enclosing an entire section>>~~ indicates that the amendment to an amendment is removing the section from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Section 36-406, Arizona Revised Statutes, is amended to
3 read:

4 36-406. Powers and duties of the department; access to
5 personnel records

6 A. In addition to its other powers and duties:

7 1. The department shall:

8 (a) Administer and enforce this chapter and the rules, regulations
9 and standards adopted pursuant ~~thereto~~ TO THIS CHAPTER.

10 (b) Review, and may approve, plans and specifications for
11 construction or modification OF or additions to health care institutions
12 regulated by this chapter.

13 (c) EXCEPT AS PROVIDED IN SUBSECTION B OF THIS SECTION, have access
14 to books, records, accounts and any other information of any health care
15 institution THAT ARE reasonably necessary for the purposes of this
16 chapter.

1 (d) Require as a condition of licensure that nursing care
2 institutions and assisted living facilities make vaccinations for
3 influenza and pneumonia available to residents ~~on-site~~ ON-SITE on a yearly
4 basis. The department shall prescribe the manner by which the NURSING
5 CARE institutions and ASSISTED LIVING facilities ~~shall~~ MUST document
6 compliance with this subdivision, including documenting residents who
7 refuse to be immunized. The department shall not impose a violation on a
8 licensee for not making a vaccination available if there is a shortage of
9 that vaccination in this state as determined by the director.

10 2. The department may:

11 (a) Make or cause to be made inspections consistent with standard
12 medical practice of every part of the premises of health care institutions
13 ~~which THAT~~ are subject to ~~the provisions of~~ this chapter as well as those
14 ~~which THAT~~ apply for or hold a license required by this chapter.

15 (b) Make studies and investigations of conditions and problems in
16 health care institutions, or any class or subclass thereof, as they relate
17 to compliance with this chapter and rules, regulations and standards
18 adopted pursuant ~~thereto~~ TO THIS CHAPTER.

19 (c) Develop manuals and guides relating to any of the several
20 aspects of physical facilities and operations of health care institutions
21 or any class or subclass thereof for distribution to the governing
22 authorities of health care institutions and to the general public.

23 B. THE DEPARTMENT'S ACCESS TO THE PERSONNEL RECORDS OF A NURSING
24 CARE INSTITUTION IS LIMITED TO THE FOLLOWING INFORMATION FOR EACH
25 PERSONNEL MEMBER, EMPLOYEE OR VOLUNTEER:

26 1. THE INDIVIDUAL'S NAME, DATE OF BIRTH AND CONTACT TELEPHONE
27 NUMBER.

28 2. THE INDIVIDUAL'S STARTING DATE OF EMPLOYMENT OR VOLUNTEER
29 SERVICE AND, IF APPLICABLE, THE ENDING DATE.

30 3. DOCUMENTATION OF ALL OF THE FOLLOWING AS PERTAINING TO THE
31 INDIVIDUAL:

32 (a) QUALIFICATIONS, INCLUDING SKILLS AND KNOWLEDGE APPLICABLE TO
33 THE INDIVIDUAL'S JOB DUTIES.

34 (b) EDUCATION AND EXPERIENCE APPLICABLE TO THE INDIVIDUAL'S JOB
35 DUTIES.

36 (c) COMPLIANCE WITH THE REQUIREMENTS OF SECTION 36-411.

37 (d) ORIENTATION AND IN-SERVICE EDUCATION AS REQUIRED BY POLICIES
38 AND PROCEDURES.

39 (e) LICENSURE OR CERTIFICATION, IF THE INDIVIDUAL IS REQUIRED TO BE
40 LICENSED OR CERTIFIED PURSUANT TO THIS ARTICLE OR POLICIES AND PROCEDURES.

41 (f) IF THE INDIVIDUAL IS A BEHAVIORAL HEALTH TECHNICIAN, CLINICAL
42 OVERSIGHT AS REQUIRED BY RULE.

43 (g) CARDIOPULMONARY RESUSCITATION TRAINING, IF REQUIRED FOR THE
44 INDIVIDUAL BY RULE.

1 (h) FIRST AID TRAINING, IF REQUIRED FOR THE INDIVIDUAL PURSUANT TO
2 THIS ARTICLE OR POLICIES AND PROCEDURES.

3 (i) EVIDENCE OF FREEDOM FROM INFECTIOUS TUBERCULOSIS, IF REQUIRED
4 FOR THE INDIVIDUAL BY RULE.

5 (j) IF THE INDIVIDUAL IS A NUTRITION AND FEEDING ASSISTANT, BOTH:

6 (i) COMPLETION OF THE NUTRITION AND FEEDING ASSISTANT TRAINING
7 COURSE AS REQUIRED BY RULE.

8 (ii) A NURSE'S OBSERVATIONS AS REQUIRED BY RULE.

9 Sec. 2. Title 36, chapter 4, article 7, Arizona Revised Statutes,
10 is amended by adding ~~[section]~~ [sections] 36-447.03 [and 36-447.04], to
11 read:

12 36-447.03. Surveys: statement of deficiencies: initiation of
13 complaint investigations; time frames; record
14 retention

15 A. [BEGINNING JULY 1, 2027.] THE DEPARTMENT SHALL ISSUE ANY
16 STATEMENT OF DEFICIENCIES TO A NURSING CARE INSTITUTION WITHIN TEN
17 BUSINESS DAYS AFTER COMPLETING THE STATE SURVEY. [NOTWITHSTANDING THE
18 DATE THAT THE STATEMENT OF DEFICIENCIES IS ISSUED, THE NURSING CARE
19 INSTITUTION SHALL COMPLY WITH THE STATEMENT OF DEFICIENCIES AND SUBMIT A
20 PLAN OF CORRECTION TO THE DEPARTMENT.]

21 ~~B. {[THE DEPARTMENT MAY NOT INITIATE A COMPLAINT INVESTIGATION~~
22 ~~RELATING TO A NURSING CARE INSTITUTION MORE THAN TWELVE MONTHS AFTER THE~~
23 ~~ALLEGED BASIS OF THE INCIDENT OCCURRED UNLESS THE COMPLAINT INVOLVES~~
24 ~~ALLEGATIONS OF CRIMINAL ABUSE OR NEGLECT]} [AND] {[FOR]} {[A FAILURE BY A~~
25 ~~PERSON WHO HAS A DUTY TO REPORT THE POSSIBLE ABUSE OR NEGLECT PURSUANT TO~~
26 ~~SECTION 13-3620 OR 46-454.]} {[IN CONDUCTING A COMPLIANCE INSPECTION OR~~
27 ~~INITIAL COMPLAINT INVESTIGATION, THE DEPARTMENT MAY NOT CONSIDER OR~~
28 ~~INVESTIGATE ANY ALLEGED VIOLATION THAT OCCURRED MORE THAN TWELVE MONTHS~~
29 ~~BEFORE THE DATE THAT THE COMPLAINT WAS SUBMITTED TO THE DEPARTMENT.~~

30 C. THE DEPARTMENT MAY INITIATE A COMPLAINT INVESTIGATION FOR
31 INCIDENTS THAT OCCURRED MORE THAN TWELVE MONTHS BEFORE THE DATE THE
32 COMPLAINT WAS SUBMITTED IF THE COMPLAINT INVOLVES AN ALLEGATION OF ABUSE
33 AS DEFINED IN SECTION 13-3623.

34 D.} IF RECORDS THAT ARE MAINTAINED PURSUANT TO ARTICLE 4 OF THIS
35 CHAPTER HAVE BEEN LAWFULLY DESTROYED PURSUANT TO RETENTION REQUIREMENTS,
36 THE DEPARTMENT MAY NOT CITE THE LICENSEE FOR FAILURE TO PRODUCE THE
37 RECORDS FOR AN INVESTIGATION INITIATED MORE THAN TWELVE MONTHS AFTER THE
38 ALLEGED BASIS OF THE INCIDENT OCCURRED.

39 ~~{[C.]}~~ {[E.]} THE DEPARTMENT SHALL INITIATE COMPLAINT
40 INVESTIGATIONS RELATING TO NURSING CARE INSTITUTIONS CONSISTENT WITH THE
41 CENTERS FOR MEDICARE AND MEDICAID SERVICES TIME FRAMES BASED ON THE
42 TRIAGED SEVERITY LEVEL OF EACH COMPLAINT.

1 36-447.04. Off-site preliminary reviews; compliance
2 inspections; complaint investigations

3 [THE DEPARTMENT MAY CONDUCT AN OFF-SITE PRELIMINARY REVIEW OF A
4 COMPLAINT OR SELF-REPORT USING WRITTEN OR VERBAL COMMUNICATIONS OR
5 DOCUMENTATION FROM THE LICENSEE TO DETERMINE WHETHER AN ON-SITE COMPLIANCE
6 INSPECTION OR COMPLAINT INVESTIGATION IS NECESSARY. IF THE REVIEW
7 CONFIRMS THAT THE LICENSEE HAS IMPLEMENTED SYSTEMS AND PROCESSES THAT MAKE
8 FURTHER VIOLATIONS UNLIKELY AND THE LICENSEE PROVIDES SUFFICIENT EVIDENCE
9 OF COMPLIANCE, THE DEPARTMENT SHALL CLOSE THE COMPLAINT AND NO FURTHER
10 INVESTIGATION IS REQUIRED.]

11 Sec. 3. Section 36-2402, Arizona Revised Statutes, is amended to
12 read:

13 36-2402. Quality assurance activities; sharing of quality
14 assurance information; immunity; confidentiality

15 A. State health care providers, hospitals and outpatient surgical
16 centers shall, and other health care entities may, conduct quality
17 assurance activities.

18 B. A health care entity may share quality assurance information
19 with appropriate state licensing or certifying agencies and with licensed
20 health care providers who are the subject of quality assurance
21 activities. A hospital may share quality assurance information with other
22 health care entities only with the approval of the hospital's medical
23 executive committee or an equivalent committee.

24 C. A health care entity may share quality assurance information
25 with other health care entities only for the purpose of conducting quality
26 assurance activities.

27 D. A health care entity or person that provides or receives
28 information, that participates, takes any action or makes any decision or
29 recommendation in the course of quality assurance activities or that
30 furnishes any records, information or assistance to a health care entity
31 for or in the course of quality assurance activities is not subject to
32 liability for civil damages or any legal action in consequence of such
33 action except as provided in section 36-445.02.

34 E. Quality assurance activities conducted by state, county or local
35 medical, pharmacy and dental associations and societies on behalf of a
36 health care entity are immune from civil liability to the same degree as
37 the facility for which the review activities are conducted.

38 F. Health care entities may jointly conduct quality assurance
39 activities.

40 G. This section does not relieve any health care entity from
41 liability arising from the treatment of a patient or from negligent
42 credentialing decisions.

1 H. LONG-TERM CARE INCIDENT REPORTS:

2 1. ARE A RISK MANAGEMENT TOOL TO BE USED SOLELY FOR INTERNAL
3 INVESTIGATIONS AND QUALITY IMPROVEMENT EFFORTS.

4 2. ARE CONFIDENTIAL RECORDS OF AN UNPLANNED EVENT OR OCCURRENCE
5 THAT IS INCONSISTENT WITH ROUTINE CARE AND THAT MAY IMPACT A RESIDENT'S
6 SAFETY, HEALTH OR WELL-BEING.

7 3. ARE USED TO SUPPORT FOLLOW-UP ACTIONS AND IDENTIFY OPPORTUNITIES
8 FOR IMPROVEMENT AND MANAGE RISK.

9 4. ARE MAINTAINED SEPARATELY FROM THE RESIDENT'S MEDICAL RECORD.

10 Enroll and engross to conform

11 Amend title to conform

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